



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : مروه خلف الله محمد محمد

العنوان : العياط

الحي / المدينة : العياط

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 29705162100346 رقم التليفون : 01095004316

العمر (تاريخ الميلاد) : 16-05-1997 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____

المقرر بما فيه : _____

التوقيع : (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب ويريدي خط الأول لعلاج

حالات ثانويات / ارتجاع أورام الثدي [14129]

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة : _____

الوظيفة : _____

الوظيفة : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة

رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / _____ أ.د. / _____ أ.د. / _____

قرار اللجنة

التشخيص النهائي : _____

خطة العلاج : _____

المستشفى : _____

قسم : _____

جهة العلاج : _____

نفقات : _____

ملاحظات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مروه خلف الله محمد محمد National ID: 29705162100346	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

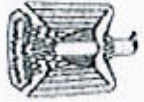
المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Trastuzumab IV

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



 جمهورية مصر العربية
 وزارة الداخلية
 مركز الجواز - الجيزة
 جواز السفر
 ٢٩٧٠٥١٦٢١٠٠٣٤٦
 JV2712905



 ٢٠٠٦/٨/٤ ٢٩٧٠٥١٦٢١٠٠٣٤٦
 جنس مصري
 الجنسية مصرية
 ٢٠٠٩/١٢/٣٠




05/03/2025

MDT FORM

Name:	Marwa Khalaf Alla Mohamed	MRN:	1903
Age:	29 Y	ID:	
Menopausal Sta:	pre	Phone:	01060048290
NRS SCORE:		Marital Status:	01095004316
BMI:		Family History:	-ve FH
CO Morbidity:	free	Reason Of MDT:	

Clinical Data:

BX:	IDC GII
ER:	8/8
PR:	8/8
HER2:	+1 tnen
KI67:	90%

R

L

Sonomammography:

Mets Workup:

History:

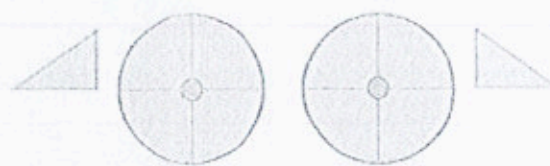
29 yrs female pt pre + ve FH >> T3 N1 >>> IDC GII ER 8/8 PR 8/8 her1 +1
>> received NACT Ac*4 TXT*4 >> rt BCS >> ypt 1c , ypn2a >>> PORT >>
on AI + zoladex since 2-2025 till now receiving verzenio 4 tnn ago till now
NECK U/S >> rt cervical LN >> biopsy of breast origin Er 4/8 Pr 0/8 her2 +3
GATA 3 +ve CT CAP 2-2026 >>> FREE
bone scan , ct cap >> R

Recommendations:

after Dr shima's consultation >> for CT CAP , bone scan , MDT

MDT Outcome & Plan

After Reviewing new CT CAP To start Taxol +Herceptin (SC if single site and IV
if metastatic).
complete 12-18 weeks of Taxol then consider shifting to ET +Herceptin

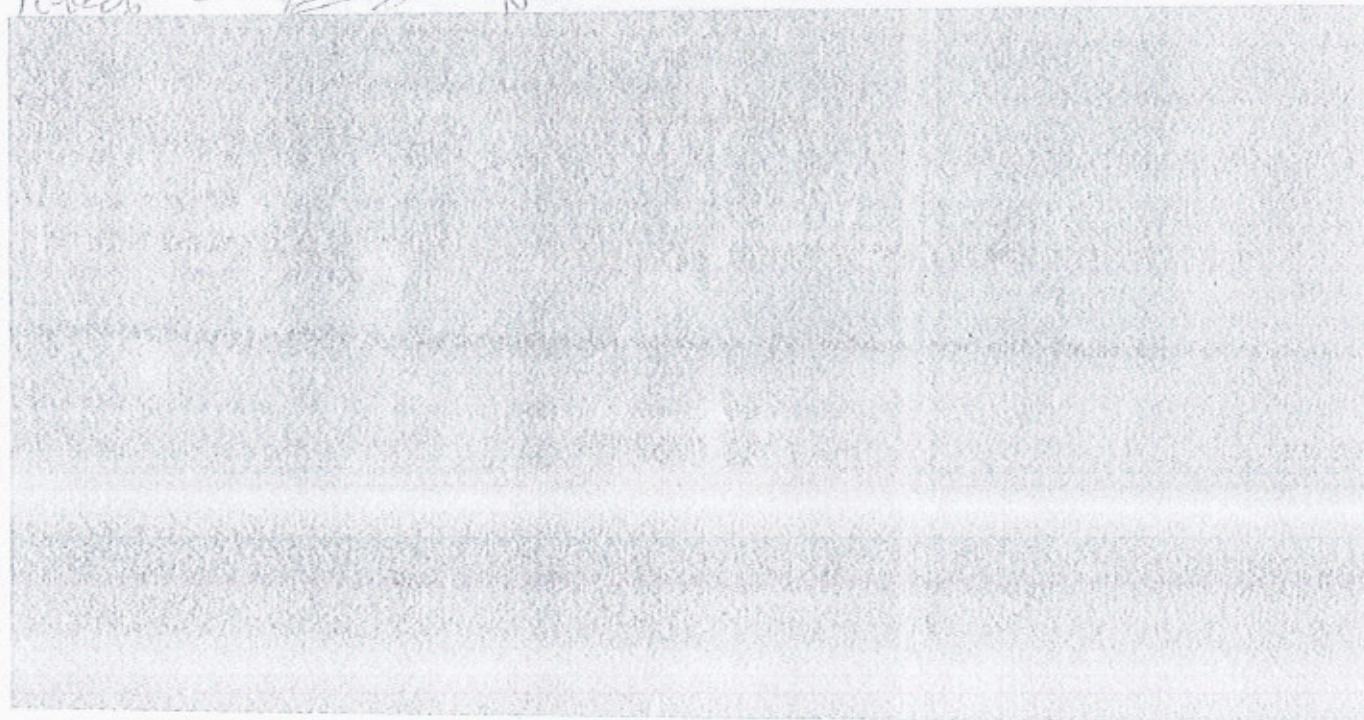


TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
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Handwritten signatures and initials in the signature box.



Name: مروه خلف الله محمد محمد		
MR. : 1903		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمناظرة وجد انه The patient presented for initiation of Zoladex, Arimidex, and supportive therapy for three months. The patient underwent neck ultrasound revealing a right supraclavicular and lower posterior cervical lymph node, which was biopsied. The patient is a 29-year-old female with a personal and family history of breast cancer. She was diagnosed with T3 N1 invasive ductal carcinoma, grade II, ER 8/8, PR 8/8, HER2 1+, GATA3 positive. She received neoadjuvant chemotherapy with 4 cycles of AC and 4 cycles of taxane, followed by right breast conserving surgery. Pathology showed ypT1c, ypN2a. She received post-operative radiotherapy and was started on AI plus Zoladex in February 2025, continuing until now with Verzenio 4 months ago. Neck ultrasound again showed right cervical lymph node involvement; biopsy confirmed breast origin with ER 4/8, PR 0/8, HER2 3+, GATA3 positive. CT chest/abdomen/pelvis in February 2026 was free of disease. The patient was cleared for CT chest/abdomen/pelvis, bone scan, and multidisciplinary team review. The patient complained of progressive neck swelling. Examination revealed a hard, fixed right cervical mass. After counseling, the patient agreed to start Taxol as soon as possible, with Herceptin to be added once available. The patient has locoregional recurrent breast cancer with right cervical lymph node metastasis of breast origin (ER 4/8, PR 0/8, HER2 3+). CT chest/abdomen/pelvis in February 2026 was free, and bone scan was free. History includes right breast cancer (T3 N1, ER 8/8, PR 8/8, HER2 negative) treated with neoadjuvant chemotherapy, right breast conserving surgery, ypT1c, ypN2a, radiotherapy, Zoladex, AI from February 2025, and Verzenio for 4 months in 2026 until June 2026, with right cervical lymph node HER2 3+. On examination, a 4 × 4 cm fungating right cervical mass and right upper limb weakness were noted. An urgent MRI of the cervical spine and brachial plexus was ordered. New CT scans were ordered for week 1 Taxol 135 mg (weight 69 kg, height 154 cm, SA 1.7). Herceptin approval was pending. Consideration was given to adding carboplatin for a more rapid response after consulting Dr. Shaimaa. For week 2 Taxol 135 mg and carboplatin AUC 2, the patient was examined with a 5 × 5 cm fungating right cervical mass and right upper limb weakness. MRI of the brachial plexus was pending. Dr. Shaimaa advised continuing Taxol and adding carboplatin, noting that the right upper limb weakness would not improve immediately. The patient has completed 2 weeks of Taxol and 1 dose of carboplatin.		
تحريراً في ,		
	التاريخ : 21-09-2026	الوقت :
Reham Ahmed Fathy Ibrahim		الطبيب المعالج :
يعتمد ,		

Patient Name: Mrwa Khalaf Allah
Mohamed

Date: 27/4/2028

^{99m}Tc -MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP;

Report:

- Degenerative changes are noticed over Dorso-lumbar vertebrae.
- No definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for referral

Nuclear Medicine Unit

DR. Sherin Mahdy, M.D

DR/ Mohamed Emam



A-Patient ID:

1- Name: مروة خالد بن محمد محمد

Sex:

3- Medical number:

Female

63188935

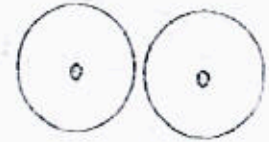
Old Med:

Old Med:

586/2024

2-ID number:

Patient Age: 28 Y



Family history

Family history
+ve

Phone

Phone
01060648290

BMI

SM

Marital status

Marital status
Single

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co morbidity
+ve

Menopausal
status

Menopausal status
Not known

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT
MDT NCT

Date of MDT

Date of MDT

04/01/2025

Breast Imaging :

Clinical data

RT BREAST UOQ MASS WITH OVERLYING AREA OF
CALCIFICATIONS 85MM PATHOLOGICAL AXILLARY LNS

Mets Workup :

CT CAP*contrast - Free

BX IDC Rx comment	ER 8/8 Rx comment	PR 6/8 Rx comment	Her 2 Neu 1 Rx comment	SISH Rx comment	KI67 90
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T 3	N 1	M 0
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Sonomammography

CESM 26/3/2024: rt UOQ pathologically proven malignant lesion BIRADS 6

PATIENT HISTORY

RT LABC , received NACT AC-->taxotere with good clinical response

MDT OUTCOME & PLAN

FOR RT BCS + ALND AFTER STEROTACTIC GUIDED LOCALISATION
(BRACKETING)AND CHEST CONSULTATION

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name: Marwa Khalaf Allah Mohamed Mohamed	Date : 06/02/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 26 Year(s)
Medical ID: 29705162100346	Pat. ID: 46681

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Report:

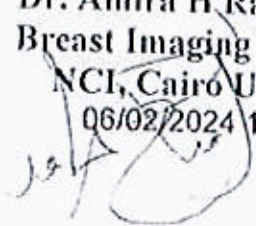
- ACR C: scattered fibroglandular parenchyma
- Right Breast:
 - Irregular dense spiculated rounded mass is seen in the UOQ measuring about 33x26x25mm with overlying segmental indeterminate microcalcification. The area of calcification and the mass are collectively measuring 68x78x85mm.
 - Ultrasound shows this mass as irregular heterogeneous isoechoic spiculated mass. It is seen at 10 to 11 o'clock, 36mm from the nipple and 2mm from the skin.
 - Multiple scattered anechoic cysts are scattered in the breast
 - About 3 pathologically enlarged level I axillary lymph node are noted showing thick cortices and effaced hila.
 - No supra or infraclavicular lymph node.
- Left Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound showed fibrocystic changes.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Right breast malignant mass & associated microcalcifications with level I axillary lymphadenopathy for histopathological assessment (BIRADS 5)
- Normal sonomammography of left breast (BIRADS 1)

Best regards,

Dr. Mariam Sabri
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
06/02/2024 1:00 PM





Total DLP (mGy.cm):

Patient Name: Marwa Khalaf Allah Mohamed Mohamed	Date : 02/03/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 26 Year(s)
Medical ID: 586/2024	Pat. ID: 46681

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

- Reported case of right breast cancer, for metastatic work-up.

I Chest:

- Right breast UOQ parenchymal soft tissue mass lesion measures about 3.8 x 2.4 cm along its axial dimensions.
- Enlarged ipsilateral axillary level I lymph nodes, largest measures about 1.7 x 1.1 cm along its axial dimensions.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.

II Abdomen & pelvis:

- The liver is of average size showing homogenous parenchymal attenuation. No definite focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys and suprarenals glands, IVC and aorta.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- Minimal deep pelvic free fluid density, likely physiological.

III The Examined Skeleton:

- No definite worrsiome lytic or sclerotic leisons.



*Patient Name: Mrwa Khalaf Allah
Mohamed*

Date: 27/4/2028

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- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for referral

Nuclear Medicine Unit

DR. Sherin Mahdy, MCD

DR/ Mohamed Emam

Patient Name	: الأستاذة / د. د. حنانة الله محمد محمد	Request No.	: 12706273
Patient No.	: 29705162100348	Request Date	: 16/02/2024
Path No.	: 549/24	Verification Date	: 14/02/2024
Sex	: Female	Financial Category	: وزير داو السلام - ميسرة
Age	: 26 Y	Department	: Out Patient
Referring Doc.		Registered by	: marwa mohamed youssef

Pathology Report**Clinical Data:** Right breast malignant looking solid mass lesion.**Nature of Specimen:** Tru-cut biopsy, collected at 6/2/2024.**Gross Examination:**

Multiple grayish white and yellow tissue cores up to 1.5 cm, totally submitted.

Histoscopic Examination:

Histological type: Invasive duct carcinoma, focally necrotizing

Tubule (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.

Ductal carcinoma in situ: Negative.

Axillary invasion: Negative.

Lymphovascular: Negative.

Diagnosis:

Right breast malignant looking solid mass lesion, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Lubna Omar

Dr. Menna Nabii.

Verified by :

Dr. Sagda El-Kady

Printing Date : 20/02/2024 10:43:15

Page 1 of 1

Printing By : marwa mohamed youssef

Patient Name : الأستاذ / مروة خلف الله محمد محمد	Request No. : 13004229
Patient No. : 29705162100346	Request Date : 15/01/2025
Path No. : 199/25	Verification Date : 29/01/2025
Sex : Female	Financial Category : أورام دار السلام - مياطرة
Age : 27 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature Specimen: Right breast conservative surgery and Axillary lymph node dissection

Gross Examination:

- * Right ALND: A fibro fatty tissue piece measured 8x7 cm entangling 10 nodules up to 1x1 cm.
- * Right BCS specimen measured 10x7 cm, with covering skin ellipse measured 5x2 cm
- * Tumor extent: Two wired firm grayish white tumor bed sites.
- * Size: 1st: 1.3x1 cm, 2nd TBS a tan nodule: 1x1 cm. Another rubbery grayish white area measured 5x4 cm.
- * Skin involvement: Unremarkable grossly.
- * Distance from margins: 0.1cm from the inferior margin.
- * Frozen section results:
- Free all margins but the inferior margin is suspicious for DCIS.

Designation:

- 1- The superior margin.
 - 2- The inferior margin.
 - 3- The lateral margin.
 - 4- The medial margin.
 - 5- The deep margin.
 - 6- The extra-inferior margin.
 - 7- The second wired nodule.
 - 8- The first wired area.
 - 9-10- The rubbery G.W area.
 - 11-12- ALND.
- Dissected by: Dr. Heba Hassan.

Verified by :

Dr. Toka Medhat

Printing Date : 04/02/2025 11:02:33

Page 1 of 3

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mide El Qadema - Cairo, Egypt

Patient Name :
Patient No. : 29705162100346
Path No. : 549/24
Sex : Female
Age : 26 Y
Referring Doc :

Request No. : 15005273
Request Date : 07/02/2024
Verification Date : 08/02/2024
Financial Category :
Department : Out Patient
IHC No. :
Registered by : marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast malignant looking solid mass lesion.
Relevant Specimen: Tru-cut biopsy, collected at 6/2/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against
- ER clone (1E2) PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9)

Results:

- ER: Positive control showed appropriate reactivity;
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8)
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+)
- Ki67: 80% nuclear reaction of tumor cells.

Conclusion:

Right breast malignant looking solid mass lesion, Tru-cut biopsy.
Invasive duct carcinoma, Grade III,

- ER: Positive (score 8/8).
PR: Positive (score 7/8).
Her2: Negative (score 1+).
Ki67: Positive in 80% of tumor cells.

فريق
الأمراض

* Scoring system: *ER, PR*
ASCO CAP ER/PR Test Guideline Recommendations, 2010 Allred score (proportion of cells staining 3+)

Verified by:
DR Menna Nabil

Printing Date: 21/02/2024 10:40:31

Page 1 of 2

Printed by: marwa mohamed youssef

Patient Name: الأستاذة / مروة محمد محمد
Patient No: 29705162100346
Path No: 549/24
Sex: Female
Age: 26 Y
Referring Doc:

Request No: 3005273
Request Date: 07/02/2024
Verification Date: 18/02/2024
Financial Category: رواتب - خدمات
Department: Out Patient

Registered by: marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: 'Her2'

Refer to: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Comment: Although Her2 is Negative. Such cases (Score 1+) are predictive of response to Her2-ant drug conjugates (ADCs) according to latest 2023 ASCO-CAP guidelines.

Signature
Prof. Dr. Lubna Omar
Dr. Taka Medhat

Verified by:
DR. Menna Nabil

Printing Date: 20/02/2024 16:43:11

Page 2 of 2

Printed By: marwa mohamed youssef

Medical No. :

10239

Patient's Name:

Marwa Khalaf Allah Muhammad
Muhammad

Date:

2026-09-27

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Examination: CT CHEST, ABDOMEN, AND PELVIS FOLLOWING IV CONTRAST, WITH
MULTIPLANAR REFORMATTED IMAGES.

Findings:

Chest:

- Well-defined, homogeneous, unenhanced, exophytic soft tissue mass in the right supraclavicular fossa measuring 3.8 x 3.6 x 3.4 cm (AP x CC x TV) with an average attenuation of approximately 36 HU.
- A few peripheral fibroatelectatic bands are noted in the right middle lobe. Otherwise, the lungs are clear without focal pulmonary nodules, consolidation, or pleural effusion.



- Trachea and mainstem bronchi are clear. No pathologically enlarged mediastinal or axillary lymph nodes. Heart size is normal. No pericardial effusion
- No suspicious osteolytic or osteoblastic osseous lesions

Abdomen:

- Liver, gallbladder, spleen, pancreas, adrenal glands, and kidneys show normal shape and attenuation without focal lesions or abnormal enhancement.
- Bowel loops demonstrate normal caliber without wall thickening, obstruction, or dynamic air-fluid levels.
- No pelvic or retroperitoneal lymphadenopathy. No free fluid or intra-abdominal free air.
- Abdominal aorta and its main branches are intact.
- No suspicious osteolytic or osteoblastic osseous lesions.

Conclusion:

1. Right Supraclavicular Soft Tissue Mass, suspicious for metastatic lymphadenopathy given the clinical history. Ultrasound-guided tissue sampling or PET/CT correlation is recommended.
2. Right middle lobe minor peripheral fibroatelectatic bands
3. No evidence of distant chest or abdominopelvic visceral or osseous metastasis.

DR.MAI.MOUHAMED